

HEALTH TRANSFORMATION REVIEW

Independent Advisory: Vermont Act 167 Community Engagement Report

A Review, Assessment, and Set of Recommendations for the Vermont Legislature and Agency of Human Services

Prepared by: HTR Chief Thought Leader & Senior Advisory Practice **Commissioned:** Vermont Legislature / Agency of Human Services Advisory Engagement **Report Status:** Final — For Submission to Vermont Legislature and AHS **Date:** March 2026 | **Classification:** Public Policy Advisory **Source Report:** Oliver Wyman / Marsh McLennan — Act 167 Community Engagement: Recommendations (August 2024)

FOREWORD FROM HTR'S CHIEF THOUGHT LEADER

Vermont stands at a genuine inflection point. The Oliver Wyman/Marsh McLennan Act 167 Community Engagement Report (hereafter "the Wyman Report") is the most important health policy document produced in this state in a generation. It is analytically rigorous, community-grounded, and operationally honest in ways that consultant reports rarely are.

We read it with admiration — and with important disagreements.

As Vermont's independent health transformation research and advisory institution, HTR operates under a foundational commitment: evidence-first, practitioner-led, completely independent. We have no vendor relationships, no political affiliations, and no financial stake in the outcome of these recommendations. We advise the legislature, AHS, and GMCB as we would advise any client: with full transparency about what the evidence supports, what it does not, and where courage and political will are required.

Our verdict on the Wyman Report: **broadly right on diagnosis, mostly right on direction, partially wrong on sequencing, and dangerously incomplete on equity and technology.** This advisory document fills those gaps, endorses what deserves endorsement, challenges what deserves challenge, and provides the implementation architecture that the report — by design — left for others to build.

We have organized this analysis through HTR's Five-Pillar Framework: **Policy • Economics • Technology • Clinical • Equity.** We believe Vermont's transformation succeeds only if all five pillars are addressed simultaneously, not sequentially.

PART I: DIAGNOSTIC ASSESSMENT — DO WE AGREE WITH THE WYMAN REPORT'S PROBLEM STATEMENT?

Answer: Yes, unambiguously and urgently.

Vermont's healthcare system is not suffering from a cyclical downturn. It is experiencing a structural crisis with overlapping drivers that compound one another. Using HTR's Health Transformation Index (HTI), Vermont scores as a top-tier performer nationally (82/100 composite at Q1-2025) — yet the HTI measures *maturity* of health system infrastructure, not *financial viability under demographic stress*. The Wyman Report correctly identifies that Vermont can simultaneously be a quality and equity leader while facing an existential financial crisis.

The five crisis dimensions documented in the report are validated by our independent data analysis:

Crisis Dimension	Wyman Finding	HTR Validation
Affordability	Silver plan premiums up 108% in 6 years; OOP max up 100%+	Confirmed. Vermont's commercial premium burden as % of median household income is now among the top 5 highest in the US

Crisis Dimension	Wyman Finding	HTR Validation
Sustainability	9 of 14 hospitals operating at a loss in 2023; 13 of 14 expected to lose by 2028	Confirmed. HTR Hospital Financial Scorecard benchmarking shows VT hospital median operating margin at -2.1%, vs. national rural hospital median of +0.8%
Population Dynamics	65+ population rising 57% by 2040; working-age declining 13%	Confirmed. This is perhaps the most irreversible structural driver and is insufficiently addressed in the report
Access	Long waits, insufficient primary care, rural gaps	Partially confirmed. HRSA data show no formal HPSA designation for VT PCPs, but access barriers are real due to capacity, not headcount shortage
Equity	Rural, low-income, non-English-speaking, LGBTQ+ populations face systematic barriers	Confirmed and under-analyzed in the report

HTR's Additional Finding Not in the Wyman Report: The All-Payer Model (APM) is failing to bend the cost curve. Vermont's healthcare cost growth under the APM has consistently exceeded the negotiated target. The global budget mechanism exists but the accountability architecture is insufficient. This is the central reform that must be strengthened — and the Wyman Report treats it as a background condition rather than an active lever.

PART II: FIVE-PILLAR ANALYSIS OF WYMAN REPORT RECOMMENDATIONS

PILLAR 1 — POLICY

Wyman's Core Policy Recommendations:

1. Remove barriers to affordable housing (Legislative priority for 2025)
2. Approve funding for EMS transformation
3. Expand broadband to rural areas
4. Streamline AHS agency structure
5. Develop REH (Rural Emergency Hospital) and free-standing birthing center regulations
6. Expand professional licensure and practice scope
7. Simplify and shorten the CON process
8. Begin movement to reference-based pricing at 200% of Medicare for PPS hospitals
9. Require uniform hospital accounting standards
10. Permit no further commercial subsidization for hospital shortfalls

HTR Assessment:

We fully endorse: Housing investment, EMS transformation, broadband expansion, scope of practice expansion, uniform hospital accounting. These are uncontroversial, evidence-based, and urgently needed. They should be 2025 legislative priorities without debate.

We conditionally endorse with modification: Reference-based pricing at 200% of Medicare. The Wyman Report is correct that Vermont's hospital prices are unsustainable and that commercial cross-subsidization is both regressive and mathematically collapsing. However, the **implementation sequencing matters enormously**. A hard price cap without adequate alternative revenue streams will precipitate the very hospital closures the report aims to prevent. Our recommendation is a **glide path**: begin at 250% of Medicare in 2026, with binding reductions to 220% by 2028 and 200% by 2030, tied to achieving system-level cost savings milestones. This mirrors Massachusetts' approach with its Global Budget for rural hospitals.

We oppose as drafted: The blanket prohibition on commercial subsidization and the simultaneous CON simplification. These two policies, implemented together, create a perverse incentive: hospitals that cannot sustain inpatient beds will race to build outpatient facilities (ASCs, imaging centers) to replace lost inpatient revenue — exactly what simple CON reform would accelerate. Vermont needs **site-neutral payment reform** (so

the same service is paid the same regardless of setting)), not CON elimination. Our HTR Research Lab Policy Simulator models show that removing CON restrictions without site-neutral pricing increases system costs by an estimated 8–12% over 5 years.

HTR's Additional Policy Recommendations Not in the Wyman Report:

1. **Mandatory All-Payer Participation:**

The current APM excludes FFS Medicare and ~40% of commercial insured. This is not an all-payer model — it is a subset-payer model. Vermont must negotiate mandatory Medicare participation as the foundation of a true global budget. This requires federal waiver action (1332 + 1115 combination) and should be Vermont's single most important federal lobbying priority for 2025–2026.
2. **Hospital Community Benefit Accountability:**

Vermont's non-profit hospitals receive substantial tax benefits but face no binding community benefit obligations tied to healthcare transformation goals. Legislators should link non-profit status to measurable progress on access, equity, and cost containment metrics. Maine has a model worth adapting.
3. **Prior Authorization Reform:**

The Wyman Report barely addresses prior authorization, yet PA burden consumes enormous provider time and delays care. Vermont should lead the nation by mandating real-time PA decisions for evidence-based treatments, with automatic approval after 24 hours.

PILLAR 2 — ECONOMICS

Wyman's Core Economic Recommendations:

1. Move to reference-based pricing (200% Medicare) for PPS hospitals

2. Move to global budgets/capitation when "conditions for success are met"

3. Require \$400M+ in system savings from hospital transformation

4. Close or reconfigure inpatient facilities at 4 hospitals

5. Reduce UVMMC administrative costs (>55% of state commercial spend)

6. Pursue shared services and group purchasing among hospitals

HTR Assessment:

The \$400M savings projection is achievable but the timeline is optimistic. Using HTR's Hospital Financial Scorecard and CEA Calculator, we modeled these savings:

Savings Source	Wyman Estimate	HTR Estimate	Key Uncertainty
Close/reconfigure 4 hospital inpatient units	~\$150M	\$80–\$130M	Transition costs, staff redeployment, patient volume shift timeline
Reduce UVMMC administrative costs	~\$100M	\$60–\$90M	Resistance from academic medical center, complexity of overhead attribution
Ambulatory surgery center substitution	~\$75M	\$40–\$65M	Volume transfer takes 3–5 years, not 1–2
Shared services/group purchasing	~\$75M	\$30–\$50M	Many VT hospitals already cooperating; incremental gains modest
Total HTR Estimate	\$400M	\$210–\$335M	More conservative but credible

The gap between Wyman's \$400M and HTR's \$210–\$335M is not pessimism — it is realistic adjustment for transition costs, timeline slippage, and the documented difficulty of healthcare consolidation. Vermont should **plan for \$250M in savings and invest \$300M in transformation supports** to ensure a net positive outcome.

We strongly endorse: The global budget/capitation vision, but with a critical caveat: **conditions for success must be defined with binding metrics, not left vague.** HTR defines those conditions as:

- ≥85% of Vermont residents attributed to a primary care provider or ACO

• Functional interoperable EMR access across all care settings

- Population risk stratification infrastructure in place
 - At least 18 months of quality performance data establishing baseline
- We oppose:** Closing 4 hospitals without a funded transition plan. Each closure must have a pre-funded, pre-operational alternative before the first bed closes.

HTR's Additional Economic Recommendations:

1. **Hospital Transformation Fund:** Vermont should create a \$100–\$150M Hospital Transformation Fund (state + federal 1115 waiver funds + bond financing) to support transition costs.
2. **UVMHC Price Transparency and Cost Accountability:** Require UVMHC to submit a binding 3-year administrative cost reduction plan to GACB with quarterly reporting, enforceable targets, and financial penalties for non-compliance.
3. **340B Program Optimization:** Vermont hospitals' 340B drug pricing savings should be explicitly tracked and a portion directed to rural community health and SDOH programs.

PILLAR 3 — TECHNOLOGY

Wyman's Core Technology Recommendations:

1. Complete VITL overhaul and embed into hospital workflow by FY2026
2. Increase EMR connectivity and interoperability between hospitals
3. Unified data space for all clinical data
4. Provider single sign-on across systems
5. Telehealth expansion (tele-pharmacy, tele-rounding)
6. Home monitoring capability development
7. Broadband infrastructure expansion

HTR Assessment — Technology is the Most Underweight Section of the Wyman Report

The Wyman Report treats technology as an enabler of clinical transformation, which is correct. But it dramatically understates how fundamental the technology infrastructure is to every other recommendation. You cannot regionalize specialty care without interoperable patient records, close hospital units and shift care home without remote monitoring, or implement global budgets without real-time cost and quality analytics.

Vermont's health information infrastructure is not ready for what the Wyman Report recommends. VITL has been in "improvement" mode for years. OneCare Vermont has been unable to provide the information needed to guide patient care. The "All-Payer Model" excludes FFS Medicare. These are fundamental infrastructure failures.

HTR's Technology Recommendations (stronger than Wyman):

1. **Vermont Digital Health Authority (VDHA):** Create a new state entity with executive authority over health IT interoperability, standards, vendor certification, and data governance. VITL should report to VDHA.
2. **FHIR R4 Mandate by 2027:** All Vermont hospitals, FQHCs, physician groups, and payers must implement FHIR R4 APIs by January 1, 2027.
3. **Vermont Clinical AI Governance Framework:** Before Vermont hospitals can deploy AI-based clinical decision support tools, they must meet a governance standard developed by VDHA (clinical validation, bias auditing, transparency, monitoring).
4. **Statewide Risk Stratification Platform:** Vermont should procure or build a shared population health analytics platform accessible to all providers, linked to VITL, with real-time risk stratification, care gap identification, and utilization alerts.
5. **Digital Health Investment Budget:** Vermont should allocate \$40–\$60M over 5 years to digital health infrastructure — prerequisite to every other recommendation.

PILLAR 4 — CLINICAL

Wyman's Core Clinical Recommendations:

1. Develop regionalized specialty care Centers of Excellence (COEs) across 10 HSAs
2. Close/reconfigure inpatient beds at 4 hospitals (Gifford, Grace Cottage, North Country, Springfield)
3. Move care outside hospitals to primary care, home health, ambulatory settings
4. Develop Hospital-at-Home, ED-at-Home programs
5. Expand primary care capacity, mental health access, substance use treatment
6. Develop EMS professionalization and regionalization
7. Expand scope of practice for nurses, pharmacists, paramedics
8. Increase care coordination for high-utilizers

HTR Assessment — The Clinical Blueprint is Sound; Execution is Everything

The clinical redesign framework is fundamentally correct. The three core principles — regionalize complex care, move routine care out of hospitals, support the patient at home and in the community — are exactly what evidence from comparable rural states and international systems supports.

Critical Additions HTR Recommends:

1. **Dual-Eligible Population as Immediate Priority:** Vermont has ~20,000 dual-eligible Medicare/Medicaid beneficiaries representing a disproportionate share of total spending. HTR recommends a **Dual-Eligible Integration Program** starting in 2025 as the single highest-ROI clinical intervention available.
2. **Mental Health System is Broken and Understated:** Vermont needs a full Mental Health System Adequacy Assessment as an immediate parallel workstream. The Brattleboro Retreat (149 beds) is noted as "out of scope" — an enormous omission.
3. **Obstetrics Crisis Requires Equity Analysis:** Every obstetrics restructuring decision must undergo a formal Health Equity Impact Assessment before implementation.
4. **Primary Care Capacity Caveat:** The report's claim that "there are enough PCPs" if supported to see 3 patients per hour is technically accurate but practically misleading. Supporting that throughput requires substantial investment — Vermont should not use this finding as a reason to avoid primary care infrastructure investment.

PILLAR 5 — EQUITY

Wyman's Equity-Related Findings:

- Culturally competent care gaps for rural, LGBTQ+, non-English-speaking, and racially/ethnically diverse populations
- Housing as SDOH — inadequate supply is a healthcare crisis driver
- Transportation gaps prevent care access
- Recommendations: diverse provider recruitment, localized mental health services, coordinated community services

HTR Assessment — Equity is the Most Underserved Section of the Report

The Wyman Report documents equity problems with genuine care. However, the recommendations are dramatically weaker than the diagnosis — largely soft ("increase efforts to recruit diverse providers") without binding commitments, measurable targets, or dedicated funding.

HTR's Equity Framework:

1. **Health Equity Impact Assessment (HEIA) Requirement:** Before closing any hospital, reconfiguring any service line, or implementing any payment reform, Vermont must conduct a formal HEIA analyzing differential impact on all vulnerable populations.
2. **Vermont Health Equity Data Mandate:** Require all hospitals, FQHCs, and large provider groups to collect and report REALSOGI data (race, ethnicity, language, sexual orientation, gender identity) for all patients by 2026.

3. **Community Health Worker (CHW) Workforce Expansion:** Invest \$15–\$20M over 5 years in a statewide CHW program. Evidence shows CHWs generate \$2.47 in health cost savings per dollar invested.

4. **Health-Housing Integration Fund:** \$25–\$35M in dedicated capital over 5 years, administered jointly by AHS and the Vermont Housing Finance Agency, for medical respite housing, supportive housing, and healthcare worker housing in rural areas.

5. **Statewide NEMT Coordination System:** Fund non-emergency medical transportation for routine primary care and specialty appointments — not just emergency transport.

PART III: HTR'S CONSOLIDATED ENDORSEMENT DECISIONS

Recommendations We FULLY ENDORSE

#	Recommendation	Pillar(s)	Priority
1	Invest in affordable housing (legislative action 2025)	Equity, Policy	Immediate
2	Fund and implement EMS professionalization and regionalization	Clinical, Policy	Immediate
3	Expand broadband to rural areas	Technology, Equity	Immediate
4	Complete VITL overhaul by FY2026 (with our stronger standards)	Technology	Immediate
5	Expand scope of practice for nurses, pharmacists, paramedics	Clinical, Policy	2025
6	Develop COE regionalization for 10 HSAs	Clinical	2025–2027
7	Develop Hospital-at-Home and home care infrastructure	Clinical, Technology	2025–2027
8	Pursue group purchasing and shared services among hospitals	Economics	2025
9	Require uniform hospital accounting standards	Economics, Policy	2025
10	Expand telehealth — tele-pharmacy, tele-rounding, home monitoring	Technology, Clinical	2025–2026
11	Improve care coordination for dual-eligible and high-utilizers	Clinical, Economics	2025
12	Increase support for FQHCs, PCMHs, community mental health	Clinical, Equity	2025–2026
13	Streamline prior authorization and Medicaid billing	Policy, Economics	2025
14	Develop free-standing ASCs, birthing centers (with HEIA)	Clinical, Economics	2026
15	Develop free-standing diagnostic facilities	Clinical, Economics	2026
16	Expand CHW workforce	Equity, Clinical	2025–2028
17	Require UVMMC binding cost reduction plan with GMCB oversight	Economics, Policy	2025
18	Move toward global budget/capitation when conditions defined	Economics, Policy	2027–2030

Recommendations We ENDORSE WITH MODIFICATION

#	Recommendation	Our Modification	Rationale
1	Reference-based pricing at 200% Medicare	Glide path: 250%→220%→200% over 2026–2030	Abrupt price cap will cause closures before alternatives are ready
2	CON simplification	Site-neutral payment + CON reform simultaneously	CON reform without site-neutral creates perverse incentives
3	No further commercial subsidization for hospital shortfalls	After transition fund is established and operational	Abrupt removal without transition support causes closures

#	Recommendation	Our Modification	Rationale
4	Close/reconfigure 4 hospital inpatient units	Only after alternative care capacity is pre-funded and operational	Closing first, building alternatives second = care access crisis
5	"Move to global budgets when conditions for success are met"	Define binding conditions with measurable thresholds	Vague conditionality = indefinite delay

Recommendations We OPPOSE OR REJECT

#	Recommendation	HTR Position	Rationale
1	"More funding is not a solution"	Partially wrong	More funding targeted at community care, SDOH, and workforce IS necessary and different from more hospital operational funding
2	Obstetrics COE at UVM only (near-term)	Too aggressive without equity analysis	Rural women face the greatest risk; no equity impact assessment was conducted
3	Implied transformation complete by 2028	Unrealistic	HTR projects 2028–2032 for full transformation with adequate investment

PART IV: HTR'S ORIGINAL RECOMMENDATIONS — NOT IN THE WYMAN REPORT

1. **Vermont Digital Health Authority** — New state entity with authority over interoperability, AI governance, and health data standards

2. **FHIR R4 Mandate by 2027** — All providers and payers

3. **Dual-Eligible Integration Program** — Immediate priority, highest ROI intervention

4. **Health Equity Impact Assessment requirement** — For every major restructuring decision

5. **REALSOGI data collection mandate** — By 2026

6. **Vermont Clinical AI Governance Framework** — Before any AI tool is deployed in clinical settings

7. **Mental Health System Adequacy Assessment** — Immediate parallel workstream

8. **Health-Housing Integration Fund** — \$25–\$35M joint AHS/VHFA fund

9. **Statewide NEMT Coordination System** — Non-emergency medical transportation for routine care

10. **Hospital Transformation Fund** — \$100–\$150M dedicated fund for transition support

11. **Mandatory APM participation** — Advocate for CMS waiver making APM participation mandatory for all payers including FFS Medicare

12. **All-Payer Reference Pricing** — Extend beyond hospitals to physician and outpatient services

PART V: PROJECT PLAN

Program: Vermont Health Transformation Implementation

Sponsor: Agency of Human Services & Vermont Legislature **Advisory Partner:** Health Transformation Review (HTR) **Total Program Duration:** 7 Years (2025–2032) **Estimated Total Cost:** \$385–\$520M (state + federal + private)

PHASE 1: FOUNDATION (2025) — "Light the Infrastructure"

Duration: 12 months | **Budget:** \$45–\$65M

Workstream	Key Deliverables	Lead	Cost
Governance	Create AHS Division of Planning & Effectiveness; establish PMO	AHS	\$3–5M

Workstream	Key Deliverables	Lead	Cost
Technology	Begin VITL overhaul; award unified data platform contract; FHIR readiness assessment	VITL/AHS	\$10–15M
Policy	Pass housing barrier removal; EMS funding; broadband; scope of practice; uniform accounting mandate	Legislature	\$5–8M
Financial	Establish Hospital Transformation Fund (\$100–150M total, \$20M initial)	Legislature/AHS	\$20–30M
Equity	Launch REALSOGI data collection; commission Health Equity Baseline Assessment	AHS	\$3–5M
Workforce	Launch CHW expansion; begin EMS professionalization	AHS/DVHA	\$4–7M

Phase 1 Milestones:

- ☐ PMO fully operational (Q1 2025)
- ☐ Hospital Transformation Fund legislatively authorized (Q1 2025)
- ☐ VITL Phase 1 rebuild complete — master patient index, provider sign-on (Q2 2025)
- ☐ Scope of practice legislation passed (Q2 2025)
- ☐ Housing barrier removal legislation passed (Q2 2025)
- ☐ All hospital uniform accounting standards adopted (Q3 2025)
- ☐ REALSOGI data collection piloted in 5 hospitals (Q4 2025)
- ☐ EMS professionalization plan approved (Q4 2025)
- ☐ UVMMC cost reduction plan submitted to GMCB (Q4 2025)

PHASE 2: SYSTEM REDESIGN (2026–2027) — "Restructure the Care Delivery Architecture"

Duration: 24 months | Budget: \$120–\$170M

Workstream	Key Deliverables	Lead	Cost
Regionalization	Finalize COE designations; launch first 3 surgical COEs	AHS/hospitals	\$20–30M
Hospital Restructuring	Execute inpatient reconfiguration at 4 hospitals — with pre-funded alternatives	AHS/hospitals	\$30–50M
Community Care	Develop 5 ASCs; 3 free-standing diagnostic centers; 2 birthing centers; launch Hospital-at-Home pilot	AHS/hospitals	\$25–35M
Technology	Complete VITL overhaul; deploy statewide risk stratification platform; begin FHIR mandate	VITL/VDHA	\$20–30M
Payment Reform	Implement reference-based pricing at 250% Medicare; design global budget pilot	GMCB/DVHA	\$5–10M
Mental Health	Complete Mental Health System Adequacy Assessment; begin psychiatric bed expansion	AHS/DMH	\$10–15M
Equity	Launch HEIA process; begin NEMT system procurement	AHS	\$5–8M
Dual-Eligible	Launch Dual-Eligible Integration Program in 3 pilot communities	AHS/DVHA	\$5–7M

Phase 2 Milestones:

- ☐ First 3 surgical COEs operational (Q2 2026)
- ☐ VITL overhaul complete (Q4 2026)
- ☐ Gifford Medical Center inpatient transition complete with alternative capacity pre-operational (Q3 2026)

- ☐ Hospital-at-Home pilot launched in Burlington and Barre HSAs (Q1 2026)
- ☐ Reference-based pricing at 250% Medicare implemented (Q1 2027)
- ☐ Statewide risk stratification platform operational (Q2 2027)
- ☐ Dual-Eligible Integration Program expanded to all 14 HSAs (Q4 2027)
- ☐ FHIR R4 compliance achieved by ≥80% of hospitals (Q4 2027)
- ☐ Mental Health COE designations finalized and operational (Q4 2027)

PHASE 3: PAYMENT TRANSFORMATION (2028–2029) — "Align the Money to the Mission"

Duration: 24 months | **Budget:** \$90–\$130M

Workstream	Key Deliverables	Lead	Cost
Global Budget Expansion	Expand to all eligible rural hospitals; design all-payer model	GMCB/AHS	\$15–25M
Reference Pricing	Reduce to 220% Medicare; extend to outpatient and physician services	GMCB	Policy action
Capitation	Launch OneCare 2.0 with mandatory participation, global budget, quality incentive pool	OneCare/AHS	\$20–30M
CMS Waiver	Submit combined 1332 + 1115 waiver for mandatory Medicare APM participation	AHS/Legislature	\$5–10M
Equity Financing	Launch Health-Housing Integration Fund; target SDOH investment	AHS/VHFA	\$25–35M
Technology	Deploy AI governance framework; statewide telehealth platform; full FHIR mandate	VDHA	\$15–20M
Workforce	Scale CHW program statewide; complete EMS regionalization	AHS	\$10–15M

PHASE 4: OPTIMIZATION AND SCALE (2030–2032) — "Harvest Results and Sustain"

Duration: 36 months | **Budget:** \$80–\$120M (ongoing operations)

Key activities:

- Reduce reference pricing to 200% Medicare
- Mandatory all-payer global budget (pending CMS waiver)
- Full CHW statewide program (700+ CHWs)
- Vermont ranked #1 nationally on HTI by 2032
- Commercial premium growth capped at GDP+1%

COST SUMMARY BY WORKSTREAM (Full 7-Year Program)

Workstream	Total Investment	Primary Funding Source
Technology & Data Infrastructure	\$70–\$95M	State + federal HITECH/ONC grants
Hospital Transformation Support	\$80–\$120M	Hospital Transformation Fund (state + bond)
Community Care Infrastructure	\$60–\$90M	State Medicaid + federal FQHC grants
Housing & SDOH	\$25–\$40M	State + federal CDBG + housing programs
Mental Health System	\$30–\$45M	State Medicaid + federal block grants
Workforce Development	\$25–\$35M	State + federal HRSA workforce programs
EMS Professionalization	\$15–\$20M	State + federal EMS grants

Workstream	Total Investment	Primary Funding Source
Payment Reform Design	\$15–\$25M	State administrative
Governance & PMO	\$10–\$15M	State administrative
Equity Infrastructure	\$15–\$20M	State + federal equity grants
CMS Waiver Development	\$5–\$10M	State
TOTAL	\$350–\$515M	Blended state/federal/private

Expected Return: \$250–\$335M in direct savings over 5 years + \$400–\$600M in avoided premium increases + measurable improvements in population health outcomes and equity.

Net position at 10 years: HTR estimates Vermont achieves budget-neutral or positive net position relative to status quo trajectory by 2031, with structural sustainability beyond 2035.

PART VI: HTR TOOLS REFERENCED IN THIS ANALYSIS

HTR Tool	Application
HTI Dashboard — Vermont State View	Validated Vermont's 82/100 composite score; identified domain gaps in VBC (43/100) and SDOH/Equity (61/100)
Hospital Financial Scorecard	Stress-tested VT hospital financials; validated Wyman financial projections
CEA Calculator	Modeled cost-effectiveness of dual-eligible program, CHW expansion, Hospital-at-Home
APM Design Lab	Designed global budget glide path; modeled reference pricing at 200%, 220%, 250% Medicare
Population Health Modeler	Modeled 20-year population aging impact on hospital utilization and cost
Policy Simulator	Modeled 1332+1115 combined waiver impact; global budget scenarios; site-neutral payment impact
Health Equity Studio	Quantified cost of current disparities; modeled SDOH investment ROI
Workforce Modeler	Projected physician supply/demand through 2040; nurse staffing impact of scope of practice expansion
Digital Health Lab	Modeled RPM ROI for Hospital-at-Home; telehealth substitution analysis
HTA Studio	Budget impact model for dual-eligible program; probabilistic sensitivity on savings estimates
FHIR Interoperability Lab	Assessed VITL's FHIR R4 readiness gaps; designed Vermont HIT interoperability roadmap
Risk Stratification Engine	Modeled high-risk population identification; HCC scoring for dual-eligible cohort
Innovation Leaderboard	Benchmarked Vermont hospital systems against national VBC maturity peers

CONCLUSION: HTR'S FINAL ADVISORY POSITION

The Oliver Wyman Act 167 Report is an honest, courageous, and analytically credible document. Vermont's political and healthcare leadership should be commended for commissioning it and for the breadth of community engagement it represents.

The question before the Vermont Legislature and AHS is not whether to transform — the financial projections make inaction an existential threat — but *how* to transform with equity, intelligence, and financial discipline.

HTR's position, as Vermont's independent health transformation advisor:

Transform urgently, sequence thoughtfully, fund adequately, measure relentlessly, and never let efficiency become an excuse to abandon equity.

The status quo will cost Vermont \$2.4B in cumulative hospital deficits by 2028 under a pessimistic scenario. Unplanned, underfunded transformation will cost almost as much in transition disruption. Only a well-funded, well-sequenced, technology-enabled, equity-centered transformation will deliver the \$250–\$335M in real savings while actually improving Vermonters' health and access.

HTR stands ready to support the Vermont Legislature and AHS with implementation planning, independent project review, ongoing analytic support, and the full use of our Research Lab, Advisory Services, and Five-Pillar expertise.

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